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**A. Purpose:** To assure that patient and family participate in his / her healthcare decisions

**B. Scope:** Hospital wide

**C. Policy:**

1. Definitions

a. Voluntary Informed Consent - A patient's consent is informed:

When the patient has been given sufficient information so that he / she understands the nature of his / her condition, the nature and purpose of the proposed treatment, the risks and consequences of the procedure or treatment, the feasible alternative procedure or treatment and the prognosis if the procedure is not performed nor any treatment given

b. General Consent :

When the nature and probable risks of the procedure or treatment are of such a common and ordinary nature so as to be within the patient's understanding and knowledge (e.g. injections, etc.).

c. Implied Consent in a Medical Emergency:

Consent in emergencies may be implied if the condition of the patient precludes his/her ability to make a decision regarding treatment or procedures.

A medical emergency is a situation where delay for the purposes of obtaining consent may reasonably be anticipated as endangering the life of the patient or significantly increasing the harm to the patient's health.

d. Surrogate Decision Maker:

The priority order of surrogate decision makers is: spouse, adult children, parents, adult brothers or sisters, adult grandchildren, significant other (close friend). A close friend may sign the consent form only in an emergency.

**D. Policy Guidelines**:

**a. General Guidelines :**

Written "informed consent" using the relevant Consent Form shall be obtained in the following instances:

i. All procedures performed in the Operating Theatres / Delivery Rooms.

ii. Non-routine diagnostic or therapeutic procedures performed in the hospital, and not having a specific consent form. Example:

- Any procedure under any form of anesthesia
- Aspiration cytology
- Catheterization of major vessels
- Lumbar puncture
- Endoscopy of GI or respiratory tract
- Paracentesis
- Thoracocentesis
- Blood transfusion

**b. Specific Guidelines:**

i. The primary treating Consultant shall discuss in lay terms the procedure, its risks, benefits, costs (if applicable) and alternatives with the patient or the patient's surrogate decision maker. The Consultant shall document the discussion by obtaining the patient's or his surrogate decision maker's written informed consent on the appropriate form.

ii. The patient shall sign the consent form. A surrogate decision maker may sign the consent on behalf of the patient if :

- ✓ The patient is a minor (less than 18 years of age)
- ✓ The patient desires the surrogate to sign on his/her behalf.
- ✓ The patient is mentally incapable of making an informed consent.
- ✓ The patient is mentally incapacitated.
- ✓ The patient is unconscious
- ✓ The patient has received sedation within 3 hours
- ✓ The patient is physically incapable of signing the form

iii .The order of preference of "surrogate" is as follows :

| **Type of Patient** | **Authorized Person** |
|---|---|
| Adult Patient<br>(above 18 years) | Patient himself (if capable physically)<br>If Incapable or otherwise<br>Single<br>a. Parents ( Father or mother )<br>b. Brother or Sister depending upon seniority in the family and their availability at the time of obtaining consent ( if father or mother is not available or has expired ) or the relative available.<br><br>Married<br>a. Spouse or the husband ( as the case may be )<br>b. Parents (father or mother)<br>c. Brother or Sister – satisfying the conditions stated above. |
| Minor Patient<br>( under 18 years) | a. Parents (Father or Mother)<br><br>b.Brother or Sister ( if either parents are not available and if the person has crossed 18 years).For more than one eligible brother or sister available at the time of consent, consent will be obtained from the willing persons depending upon their seniority and availability.<br><br>d. In case neither of the above mentioned relatives of the patients is present , consent would be taken from the relative available at the hospital premises |

iv. In a life-threatening emergency where the patient is unconscious or unaccompanied, consent shall be implied; therefore the patient's signature is not required. In such situations, the Consultant shall document in the patient's medical record both the nature of the emergency and the inability of the patient or surrogate decision maker to consent.

v. It is the responsibility of the person obtaining the consent to ensure that the consent form shall be properly filled prior to signing.

vi. All entries shall be in ink.

vii. Any available adult who shall be identified on the form by title or relationship to the patient shall be witness to the patient's signature or the signature of the surrogate decision maker.

viii. The date and time of signing shall be clearly indicated.

ix. The consent form must be signed by the Consultant, Anesthesiologist, patient or his decision maker and the witness prior to entry into the surgical or delivery suite .

x. If the Consultant or the anesthetist's signature is not on the consent form, the procedure shall be postponed or cancelled.

xi. The decision regarding the patient's ability to make an informed consent shall be the responsibility of the Consultant.

xii. A patient or the surrogate decision maker may revoke the consent for the procedure at any time before it is carried out. In such an event, the Consultant shall discuss the procedure again and if the patient or the decision maker still wishes to revoke the consent, then the procedure shall not be carried out. The patient or decision maker shall sign a note to the effect on the signed consent form. The Consultant shall document this in the progress notes.

**c. Consent Forms:**

1. General informed consent on admission (Ref No # )
2. Informed consent for Anesthesia (Ref No # )
3. Informed consent for Surgery (Ref No #)
4. Informed consent for HIV testing (Ref No # )
5. Informed consent for blood transfusions (Ref No # )
6. Authorization for medical and / or surgical treatment and / or procedure (Ref No # )
7. Informed consent for radiology procedures (Ref No # )
8. High risk consent form (Ref No # )